

ARISE

Adolescent Reproductive Health Information Support & Empowerment

MENTAL HEALTH & DRUGS

Recognising & Avoiding Drug Abuse

Student Study Guide & Quiz Answer Key

Module:	Mental Health & Drugs
Lesson:	Recognising & Avoiding Drug Abuse
Quiz:	25 marks (10 MCQ + 5 Fill-in-blanks + 3 Short Answer)
Pass Mark:	15/25 (60%) — Certificate awarded at 60%+
Languages:	English Kiswahili Sheng

Learning Outcomes

By the end of this lesson, you will be able to:

1. Define drug abuse and identify substances commonly misused among Kenyan youth
2. Recognise the warning signs of drug abuse across behavioural, physical, school, emotional, and social dimensions
3. Distinguish between risk factors and protective factors and explain how to strengthen your own defences
4. Apply the 4-step DARE refusal model using Kenya-specific language and scenarios
5. Describe how to support a friend showing warning signs and access Kenya's drug support resources

Slide 2: What Is Drug Abuse? — Definitions & Kenya Reality

Drug abuse is not limited to hard drugs. It includes any substance used in a way that causes harm — and it is far more common in Kenya than most people admit.

Type of Use	Description	Risk
Experimental	First-time or occasional use driven by curiosity or peer pressure	HIGH — addiction begins here for many people
Recreational	Regular use in social settings — 'only weekends' or 'only with friends'	VERY HIGH — tolerance and dependence develop quickly
Dependent / Addicted	Compulsive use despite serious harm — cannot stop without help	CRISIS — needs professional treatment urgently

Substance in Kenya	Common Form	Why Underestimated	Real Risk
Alcohol ■	Chang'aa, beer, wine, spirits	Legal for adults; socially normalised at events	Most abused substance among Kenyan teens; kills brain cells; liver damage; accidents; violence
Cannabis / Bhang ■	Smoked, tea, edibles	'It's natural' myth; 'everyone does it'	IQ loss, psychosis risk tripled, lung damage, addiction
Glue / Petrol Sniffing ■	Shoe glue, petrol, correction fluid	Cheap, easily available, seen as 'not a real drug'	Brain damage with first use; sudden death possible; highly toxic
Prescription Misuse ■	Cough syrups (Actifed, codeine), sleeping pills, pain medications	'It's medicine, not drugs'; available at chemists	Codeine addiction; overdose danger; respiratory depression; death

Slide 3: Warning Signs — In Yourself & Others

Recognising warning signs early — before addiction takes hold — is when intervention is most effective. These signs appear in clusters, not isolation.

Category	Warning Sign	What It Could Mean
Behaviour ■	Secretive, lying about whereabouts, stealing money, dropping old friends for new secretive ones, missing school or late nights	New drug-using social environment; funding habit; covering tracks
School ■	Sudden unexplained grade drop, increased absenteeism, not doing homework, poor concentration, losing interest in subjects they used to enjoy	Brain function impaired by substances; motivation destroyed; disengagement
Physical ■■	Red or bloodshot eyes, smell of alcohol or smoke on breath or clothing, weight loss, deteriorating hygiene, unusual sleep patterns (sleeping in class or staying up all night)	Direct physical effects of substance; self-neglect; disrupted sleep/wake cycle
Emotional ■	Dramatic unexplained mood swings, irritability when questioned, sudden aggression, periods of unusual euphoria followed by deep lows, expressing hopelessness	Brain chemistry disrupted by substance; emotional dysregulation; possible underlying mental health condition
Social ■	Withdrawing from family and close friends, giving up sports or hobbies they loved, only spending time with new secretive friends, becoming defensive about their social life	New drug-using peer group replacing old; isolation from accountability; shame and hiding

■■■ Important Note

■ Seeing ONE sign alone does not confirm drug use — it may have another explanation. But SEVERAL signs appearing TOGETHER over weeks, especially alongside secretive behaviour and withdrawal, warrant a caring conversation. Do not wait for certainty to act with concern.

Slide 4: Risk Factors vs Protective Factors

No one chooses to become addicted. Drug abuse risk is shaped by a combination of vulnerability factors and protective buffers. Understanding both helps you strengthen your defences.

Risk Factor	Why It Increases Risk	Protective Counterpart
Family addiction history	Genetic predisposition + normalised drug use seen at home from childhood	Open family conversations about drugs; clear household rules
Depression, anxiety, unresolved trauma	Substances used to self-medicate emotional pain — temporarily numbs feelings	Accessing mental health support; counsellor; Befrienders Kenya 0722 178 177
Drug-using peer group	Direct access to substances + social pressure + normalisation ('everyone does it')	Building friendships with non-users; engagement in sport/arts/faith communities
School failure and disengagement	Loss of positive identity and future orientation; too much unstructured time	Tutoring support; extracurriculars; career exploration; mentorship
Social isolation and loneliness	Substances fill the void of connection; peer groups using drugs provide false belonging	Safe social spaces (clubs, sport, faith groups, youth programmes); trusted adult relationship
Easy access to substances	Availability reduces the barrier to experimentation, especially for impulsive adolescents	Awareness of where substances are present; avoiding high-risk environments

Jayden's Turning Point — Kibera

Jayden, 15, Kibera, had a rough year: his parents separated, he failed two exams, and his old friends moved schools. By term 3, he was hanging out with older boys who drank chang'aa. His grades dropped further and he began missing school. His aunt noticed and had a quiet, caring conversation with him — not accusations, just genuine concern. She took him to the school counsellor. Jayden says: 'She didn't make me feel like a criminal. She made me feel like someone worth saving. That changed everything.'

Slide 5: Refusal Skills — Your Armour in the Moment

The time to decide you will refuse drugs is NOW — before you are in the situation. A decision made under social pressure in the moment is far weaker than one made in advance.

KEY TERM: The 4-Step DARE Refusal Model

D — Direct assertive NO ('No thanks — not for me.'). A — Avoid or offer an Alternative ('Let's do something else instead'). R — give a Reason if helpful ('I've got early morning practice'). E — Exit strategy — a pre-planned believable excuse to leave if pressure continues ('I need to get home — mum's calling'). Practise these out loud until they feel natural.

Pressure Line	Strong Response	Why It Works
'Just try it once — it won't hurt'	'No thanks — that's how every addiction started. I'm good.'	Uses facts, not emotions
'Everyone is doing it — don't be a coward'	'Not everyone — and I'm not. Smart, not scared.'	Reframes 'coward' — turns pressure around
'It will make you feel amazing'	'I feel great already — and I'll still feel great tomorrow.'	Shows the cost they are not mentioning
'Are you scared of a little bhang?'	'Not scared — I've seen what it does to IQ and lungs. Hard pass.'	Uses knowledge as confidence, not morality
'You think you're better than us?'	'I like you all — I just make different choices for myself.'	Defuses conflict without backing down

Slide 6: Module Video

Watch carefully, then continue to the scenarios.

Slide 7: Real Talk — What Would You Do?

Pick your choice and see the consequence.

Slide 8: Supporting a Friend & Getting Help in Kenya

If you recognise warning signs in a friend, your response can be the turning point in their life. The right approach is not confrontation — it is genuine, caring connection.

Step	What To Do	What NOT To Do
1. Choose the right moment	Private, calm — not in front of others, not when they are intoxicated	Confront them publicly or post about it on social media

Step	What To Do	What NOT To Do
2. Open with genuine care	'I've noticed some changes and I care about you — can we talk?' Say their name. Make eye contact.	Start with accusations, ultimatums, or 'you need to stop'
3. Listen more than you speak	Ask questions and hear the answers. Don't interrupt or judge. Acknowledge their feelings.	Lecture, moralize, or make it about your own feelings about their choices
4. Encourage professional help	'Have you considered talking to the school counsellor? I'll go with you if you want.'	Try to be their therapist or promise to fix them yourself
5. If safety is at risk — tell an adult	If their drug use is dangerous or escalating, tell a trusted teacher, parent, or counsellor. This is not betrayal — it is protection.	Promise to keep it secret at the cost of their safety

Support Resource	Contact	What They Offer	Cost
NACADA Kenya	0800 723 253	Drug abuse prevention education, counselling referrals, treatment connections, 24/7	FREE
School Counsellor	At your school	Drug concerns, mental health support, referrals — first and most accessible point	FREE
Mathare Hospital Nairobi	020 2723 031	Inpatient and outpatient addiction treatment, detox, therapy	Free with referral
Coast General Hospital	041 231 4201	Addiction unit — serving Mombasa and Coast region	Free with referral
Befrienders Kenya (if emotional crisis)	0722 178 177	Emotional support, suicide prevention — for someone using drugs who is in mental health crisis	FREE

Quiz — Answer Key

Total: 25 marks | Pass: 15/25 (60%) | Certificate at 60%+

Part A: Multiple Choice (10 marks)

1. 1. Drug abuse is BEST defined as:

- ☐ A. Taking prescribed medication exactly as directed by a doctor
- ☒ B. Using any substance — legal or illegal — in a way that harms physical, mental, or social wellbeing
- ☐ C. Drinking coffee or tea every morning
- ☐ D. Using alcohol once at a celebration

2. 2. Which combination of signs is MOST likely to indicate drug abuse in a classmate?

- ☐ A. Eating more food and getting better grades
- ☒ B. Sudden grade drop, secretive behaviour, red or bloodshot eyes, withdrawing from usual friends, money going missing, and mood swings
- ☐ C. Being very talkative and energetic only
- ☐ D. Sleeping 8 hours a night

3. 3. Peer pressure leads to drug use most powerfully through:

- ☐ A. Physical force that cannot be resisted
- ☒ B. The desire to belong, fear of rejection, wanting to appear mature, and not wanting to seem different from the group
- ☐ C. Teachers encouraging students to try substances
- ☐ D. Advertisements on television

4. 4. Which of the following is a PROTECTIVE factor against drug abuse?

- ☐ A. Having no rules or curfews at home
- ☒ B. Strong family bonds, clear personal values, positive activities like sport or faith, and a trusted adult to talk to
- ☐ C. Spending most time alone with no supervision
- ☐ D. Having friends who use drugs casually

5. 5. Which of the following is a RISK factor that increases the likelihood of drug abuse?

- ☐ A. Having a trusted school counsellor
- ☒ B. Family history of addiction, unresolved trauma or depression, school failure, drug-using peer group, and social isolation
- ☐ C. Participating actively in school sports teams
- ☐ D. Having regular meals and adequate sleep

6. 6. The MOST effective 4-step drug refusal strategy is:

- ☐ A. Run away without saying anything
- ☒ B. Give an assertive NO with a reason, offer an alternative activity, and have a pre-planned exit strategy ready
- ☐ C. Accept it once to avoid conflict then never again
- ☐ D. Explain your full life story to justify refusing

7. 7. An 'exit strategy' when facing drug pressure means:

- ☐ A. Running as fast as you can without explanation

☒ B. A pre-planned believable reason to leave a high-pressure situation — for example, 'I need to get home, my mum is calling me'

☐ C. Calling the police immediately

☐ D. Agreeing to use drugs and then spitting them out

8. 8. Which of these substances is commonly abused by Kenyan students but often underestimated as dangerous?

☐ A. Vitamins

☒ B. Alcohol, bhang (cannabis), glue/petrol sniffing, and prescription drug misuse — all widely available and socially normalised in some settings

☐ C. Herbal teas

☐ D. Fruit juice

9. 9. When supporting a friend you suspect is using drugs, the BEST first approach is:

☐ A. Tell everyone at school so they can help

☒ B. Have a private, non-judgmental conversation expressing genuine concern, listen carefully, and encourage them to speak with a trusted adult or counsellor

☐ C. Ignore it completely — it is not your business

☐ D. Take their drugs away by force to help them stop

10. 10. NACADA Kenya's free 24/7 drug abuse helpline is:

☐ A. 0722 178 177

☒ B. 0800 723 253

☐ C. 999

☐ D. 116

Part C: Short Answer (25 marks)

1. 1. Using any substance in a way that harms your health, relationships, or daily functioning is called drug _____. (3 marks)

2. 2. The desire to belong and the fear of _____ are the most powerful emotional drivers behind peer-pressured drug use. (3 marks)

3. 3. Strong family bonds, clear personal values, and positive activities like sport or faith are examples of _____ factors against drug abuse. (3 marks)

4. 4. A pre-planned believable reason to leave a high-pressure situation is called an _____ strategy. (3 marks)

5. 5. The NACADA Kenya free 24/7 drug abuse helpline number is 0800 723 _____. (3 marks)

6. Distinguish between risk factors and protective factors in drug abuse prevention. Give TWO examples of each and explain why each matters. (4 marks marks) (4 marks)

7. Describe a complete 4-step strategy for refusing drugs when pressured by a friend group. Give an example of what you would say at each step. (3 marks marks) (3 marks)

8. Your close friend has recently started using alcohol every weekend. You've noticed grade drops, mood changes, and they are now missing football practice. Write what you would do step by step — including what you would say and who you would involve. (3 marks marks) (3 marks)